

VOL. 04 · SPECIALTY SERIES

AETHERA · SPECIALTY BILLING SERIES

OB/GYN Billing Solutions

Maximizing Revenue Across Office E/M, Global OB, Surgical Procedures, Prenatal Testing, and Infertility

Prepared by: Aethera Healthcare Solutions · Lakeland, FL

Scope: CPT 59400-59618 · 99202-99215 · 57170-58900 · 76801-

Updated: 2025 Edition · aetherahealthcare.com · + 1 (863) 694-0325





ABOUT AETHERA

Maximizing Revenue. Minimizing Burden.

Aethera Healthcare Solutions is your full-service medical billing partner headquartered in Lakeland, FL. We handle coding, claims, payments, denials, and collections — so you can focus on patients.

OPERATING METRICS

500+

Practices Served
across 25+ medical specialties

\$12M+

Revenue Recovered
denied & underbilled claims recouped

95%

Clean Claim Rate
vs 84% industry avg

< 30d

Average AR
days in accounts receivable

WHY PRACTICES SWITCH

01 · DIFFERENTIATOR

Personal, Not Corporate

Dedicated account team with deep specialty/payer knowledge.

02 · DIFFERENTIATOR

Performance Targets, In Writing

96%+ net collection, <5% denial, 95%+ coding accuracy, 24h posting, 48h submission, 65%+ appeal success.

03 · DIFFERENTIATOR

Zero Risk to Start

No setup fees, no long-term contracts, 90-day cancel, free 5-day audit.



OB/GYN • MARKET CONTEXT

Why OB/GYN Billing Matters Now

OB/GYN is unique in combining office, surgical, and obstetric services under one specialty. Global obstetric packages (59400–59618) bundle all antepartum, delivery, and postpartum care into a single payment — proper documentation determines whether the practice captures full revenue.

\$ 7 8 B

US Women's Health Treatment Spend (2024)

Includes OB, GYN surgery, well-woman, infertility.



3 . 6 M

Annual US Births (2024)

~92% have insurance coverage; ~8% uninsured (Medicaid expansion gap).



~ 1 4 %

OB/GYN Claim Denial Rate (Industry Avg)

Driven by global package billing errors and modifier issues.



1 3 visits

Standard Antepartum Visit Count

For uncomplicated pregnancy — bundled into 59400 global OB package; cannot bill separately.



NET EFFECT

OB/GYN practices that master global obstetric package tracking, surgical modifier -51/-59 use, and prenatal testing (76801–76830) bundling rules can recover **6–12%** of total revenue currently lost to denials and under-billing.



OB/GYN · OVERVIEW

The Four Pillars of OB/GYN Billing

Each pillar represents a distinct service family with unique coding, global period, and payer requirements.

01 · OFFICE E/M



Office E/M & Well-Woman Visits

Office visits (99202–99215), well-woman annual (G0101, Q0091), contraception counseling, IUD insertion (58300).

CPT 99202–99215 · G0101 ·
Q0091 · 58300

02 · GLOBAL OB



Global Obstetric Packages

59400 vaginal delivery, 59510 cesarean, 59610/59618 VBAC. Bundles all antepartum, delivery, and postpartum care into single payment.

CPT 59400–59618 · 13 antepartum
visits bundled

03 · GYN SURGERY



Gynecologic Surgery

Hysterectomy (58150–58294), laparoscopy (58660–58673), LEEP (57461), hysteroscopy (58558–58565). 90-day global period; modifier -51 multiple procedures.

CPT 57170–58900 · 90-day global
· Pre-auth

04 · PRENATAL + INFERTILITY



Prenatal Testing & Infertility

Ultrasound (76801–76830), amniocentesis (59000), CVS (59015), infertility (58321–58323 IUI, 58970 IVF retrieval).

CPT 76801–76830 · 59000 · 58970 ·
Pre-auth for IVF



PILLAR 01 · OFFICE E/M

Office Visits, Well-Woman & Contraception

OB/GYN office encounters cover annual well-woman exams, contraception management, abnormal bleeding, pelvic pain, and menopause. The 2021 E/M coding reform shifted documentation to MDM-based. Well-woman exams use preventive medicine codes (99381–99397) or G0101 + Q0091 for Medicare. IUD/implant insertion has separate procedure codes.

E/M coding (2021+ reform)

Code by Medical Decision Making complexity or time; 99202–99215 for problem visits. Modifier 25 required if same-day procedure.

Well-woman annual

99381–99397 preventive medicine (commercial); G0101 (pelvic exam) + Q0091 (Pap smear) for Medicare patients. Cannot bill both preventive and problem E/M same day.

Contraception procedures

58300 IUD insertion (\$185); 58301 IUD removal (\$115); 11981 implant insertion (\$225); 11982 implant removal (\$165). Device cost separate (J7307 for IUD).



Modifier 25 misuse — appending to E/M when procedure on same day does not represent significant, separately identifiable service — is the #1 OB/GYN audit trigger.

CPT REFERENCE — TOP OFFICE CODES

Code	Service	Avg \$
99213	Est. E/M, low MDM	\$92
99214	Est. E/M, moderate MDM	\$131
99395	Preventive, est., 18-39 yrs	\$185
G0101	Pelvic & breast exam (Medicare)	\$35
58300	IUD insertion	\$185
76830	Transvaginal ultrasound	\$185



PILLAR 02 · GLOBAL OB

Antepartum + Delivery + Postpartum = Single Payment

Global obstetric packages bundle ALL routine antepartum visits, delivery, and postpartum care into a single payment. Standard antepartum visit count: 13 visits for uncomplicated pregnancy. Complications may justify separate billing with modifier -24 or -25.

GLOBAL OB PACKAGE CODES

Routine Vaginal Delivery

59400

Includes: 13 antepartum visits, routine vaginal delivery, routine postpartum care (6 weeks). Total package: ~ \$2,800. Billed once per pregnancy.

Cesarean Delivery

59510

Includes: 13 antepartum visits, cesarean delivery, routine postpartum care. Total package: ~ \$4,200. Higher reimbursement for surgical delivery.

VBAC

59610 / 59618

Vaginal birth after cesarean: 59610 routine VBAC (\$3,200); 59618 VBAC with postpartum complications (\$3,800). Higher risk; some payers require pre-auth.

Antepartum-Only

59425 / 59426

59425 antepartum only, 4–6 visits (\$425); 59426 antepartum only, 7+ visits (\$850). Used when patient transfers care mid-pregnancy.

SPECIAL-CASE BILLING

⚠️ Complications Allow Separate Billing

Pregnancy complications (HTN, GDM, preterm labor) may justify separate E/M billing with modifier -24 during global postpartum or -25 during antepartum.

😊 Postpartum-Only (59430)

Postpartum care only: \$250. Used when patient delivered elsewhere and transfers care postpartum.

👶 Multiple Gestation

Twins/triplets: bill 59400/59510 once + add-on 59409 (additional baby, \$425). Higher acuity; pre-auth may apply for high-order multiples.



PILLAR 03 · GYN SURGERY

Hysterectomy, Laparoscopy, Hysteroscopy & LEEP

GYN surgery covers hysterectomy, laparoscopic procedures, hysteroscopy, and cervical procedures. All major GYN surgery has a **90-day global period** — all related follow-up care bundled. Modifier **-51** (multiple procedures) and **-59** (distinct procedural service) are critical for same-session multiple procedures.

Hysterectomy

58150 total abdominal hysterectomy (\$2,400); **58260** vaginal hysterectomy (\$2,150); **58570** laparoscopic total hysterectomy (\$2,800); **58572** lap supracervical hysterectomy (\$2,950).

Laparoscopy

58660 diagnostic laparoscopy (\$1,250); **58661** laparoscopy with removal of adnexa (\$1,650); **58673** laparoscopic myomectomy (\$2,400); **58970** IVF retrieval (\$1,800).

Hysteroscopy

58558 hysteroscopy with D&C and biopsy (\$925); **58561** hysteroscopy with myomectomy (\$1,400); **58565** hysteroscopy with lysis of adhesions (\$1,150).



Modifier 51 (multiple procedures) reduces reimbursement 50% for second procedure same session.
Modifier 59 (distinct procedural service) bypasses bundling when procedures are genuinely separate.

SURGICAL RCM KPIS

\$2,150 – \$2,950

Hysterectomy reimbursement range



90 days

Standard GYN surgical global period



~ \$1,400

Laparoscopic myomectomy (58573)



~\$925

Hysteroscopy with D&C (58558)





PILLAR 04 · PRENATAL + INFERTILITY

Ultrasound, Amniocentesis, CVS & IVF

Prenatal testing includes ultrasound (76801–76830), amniocentesis (59000), and chorionic villus sampling (59015). Infertility services include IUI (58321–58323), IVF retrieval (58970), and embryo transfer (58974). Most infertility is NOT covered by insurance; 19 states have infertility mandates.

01 Prenatal Ultrasound

76801 ultrasound < 14 weeks \$185 ·
76805 ≥ 14 weeks, complete \$225 · 76811
detailed anatomic survey \$325 · 76815 limited
\$120 · 76830 transvaginal \$185

02 Amniocentesis & CVS

59000 amniocentesis, diagnostic \$425 · 59015
chorionic villus sampling \$650. Pre-auth may apply
for high-risk only; documentation of indication
required.

\$185–\$325 Prenatal ultrasound reimbursement range

\$425–\$650 Amniocentesis / CVS reimbursement

\$1,400–\$1,800 IVF procedure reimbursement

19 states Mandate infertility coverage (2024)

03 IUI & Sperm Procedures

58321 artificial insemination, intrauterine \$225 ·
58322 sperm washing \$115 · 89261 sperm
preparation \$95. Often cash-pay if infertility not
covered.

04 IVF Procedures

58970 IVF egg retrieval \$1,800 · 58974 embryo
transfer \$1,400 · 89250 oocyte identification
\$225 · 89251 oocyte culture \$425. High-cost; 19
states mandate coverage.

 **Infertility coverage varies by state mandate.** 19 states (AR, CA, CT, DE, HI, IL, LA, MD, MA, MT, NJ, NY, OH, RI, TX, UT, WV, NH, NV) require some infertility coverage. Verify patient coverage before scheduling IVF cycle.

OB/GYN • BILLING CODES

Top CPT Codes & 2026 National Average Reimbursement

Most-billed codes across the four OB/GYN pillars. Professional component shown.

Code	Service	Modifier	Avg \$	Notes
99213	Est. E/M, low MDM	—	\$92	Most common OB/GYN E/M
99214	Est. E/M, moderate MDM	—	\$131	Complicated antepartum visit
99395	Preventive, est., 18-39 yrs	—	\$185	Well-woman annual (commercial)
G0101	Pelvic & breast exam (Medicare)	—	\$35	Medicare well-woman
59400	Routine vaginal delivery, global	—	\$2,800	13 antepartum + delivery + postpartum
59510	Routine cesarean delivery, global	—	\$4,200	Surgical delivery
59610	VBAC, routine	—	\$3,200	Vaginal birth after cesarean
58150	Total abdominal hysterectomy	—	\$2,400	90-day global
58570	Laparoscopic total hysterectomy	—	\$2,800	90-day global
58558	Hysteroscopy with D&C and biopsy	—	\$925	Diagnostic + therapeutic
58000	Uterine artery embolization	—	\$1,000	Uterine artery embolization

MODIFIER REFERENCE • A

Modifier 25 — significant, separately identifiable E/M same day as procedure.

Modifier 51 — multiple procedures same session (50% reduction). **Modifier 59** — distinct procedural service.

MODIFIER REFERENCE • B

Modifier -26 — professional component (ultrasound interpretation). **Modifier -TC** — technical component (equipment). Global OB package (59400–59618) billed ONCE per pregnancy.

Top 8 OB/GYN Denial Reasons & How Aethera Prevents Each

OB/GYN denial rates average 12-16%; each denied claim costs \$40-100 to rework. Global OB package billing errors and infertility coverage verification drive 50%+ of denials.

RED Global OB package double-billing

24 % Root cause: Antepartum visit billed separately when already bundled into 59400 global package
 Aethera global OB tracker auto-flags bundled services during pregnancy

RED Infertility not covered

18 % Root cause: IVF/IUI billed to insurance without verifying state mandate coverage
 Aethera infertility coverage verification at scheduling; cash-pay workflow for non-mandate states

AMBER Well-woman miscoding

14 % Root cause: G0101 + Q0091 billed for non-Medicare patient; should use 99381-99397 commercial
 Aethera payer-specific well-woman coding rules

AMBER Modifier 25 documentation insufficient

11 % Root cause: E/M + procedure same day without separate documentation supporting E/M significance
 Aethera modifier 25 documentation checklist before claim release

AMBER Multiple procedures modifier missing

9 % Root cause: Multiple GYN procedures same session without modifier 51; second procedure denied
 Aethera modifier 51 auto-append for multiple procedures same session

GREEN Prenatal ultrasound bundling

7 % Root cause: Multiple prenatal ultrasounds billed without medical necessity supporting additional scans
 Aethera prenatal ultrasound documentation checklist

GREEN IUD insertion without device code

6 % Root cause: 58300 billed without J7307 device; device cost lost
 Aethera device pairing rules engine requires J-code with procedure

GREEN VBAC pre-auth missing

5 % Root cause: 59610 VBAC performed without commercial payer pre-auth; full denial
 Aethera PA tracker auto-submits for VBAC scheduling



**CUMULATIVE
IMPACT**

Aethera reduces OB/GYN denial rate from industry avg **14 %** to **< 5 %** — recovering **~\$140K/year** per **\$1.5M** practice revenue.



CASE STUDY · INTEGRATED OB/GYN RCM

5-OB/GYN Practice — Annual Revenue Impact

A \$4.2M-revenue practice models the combined upside of global OB tracking, modifier 51/59 discipline, and infertility coverage verification.

01 · GLOBAL OB TRACKING Global OB Tracking

- Annual deliveries 480
- Pre-Aethera global OB double-billing rate 14%
- Post-Aethera rate 3%
- Recovered / reduced-denied claims 53 / yr
- Avg claim value \$1,400

ANNUAL REVENUE RECOVERED

\$74,200

02 · MODIFIER DISCIPLINE Modifier 51/59 Discipline

- Annual GYN surgical cases 320
- Pre-Aethera modifier denial rate 18%
- Post-Aethera rate 5%
- Recovered claims 42 / yr
- Avg surgical value \$1,850

ANNUAL REVENUE RECOVERED

\$77,700

03 · COVERAGE RECOVERY Infertility Coverage Recovery

- Annual IVF cycles 200
- Pre-Aethera infertility coverage denial rate 24%
- Post-Aethera (verification + appeals) 8%
- Recovered cycles 32 / yr
- Avg IVF cycle value \$3,200

ANNUAL REVENUE RECOVERED

\$102,400

Combined annual revenue
recovery & ROI

RECOVERED
\$254,300

YEAR-1 CC
~\$96K

YEAR-1 N
\$158,300

ROI
165%



Where OIG, Medicare RAC, and Commercial Payers Audit First

Top compliance risks across the four OB/GYN pillars. Quarterly self-audits recommended.

+ 0 Office E/M	✦ 0 Global OB	✂ 0 GYN Surgery	m 0 Prenatal + Infertility
RED Well-woman miscoding Billing G0101 + Q0091 for non-Medicare patient (should use 99381–99397 commercial); automatic denial on audit.	RED Global OB double-billing Billing separate E/M for routine antepartum visit when already bundled into 59400 global package; automatic denial.	HIGH Modifier 51 missing Multiple GYN procedures same session without modifier 51; second procedure denied at 100%.	RED IVF without coverage verification Billing IVF to insurance in non-mandate state; full denial + patient balance bill risk.
AMBER Modifier 25 misuse Appending to E/M when IUD insertion or biopsy on same day does not represent significant, separately identifiable service.	AMBER Complications without modifier 24/25 Billing separately for pregnancy complication without proper modifier; bundled and denied.	MEDIUM Modifier 59 misuse Using 59 to unbundle procedures that should be bundled based on same anatomic site/session.	AMBER Prenatal ultrasound w/o necessity Billing multiple 76805 ultrasounds without supporting documentation; reduced to 1 per pregnancy without medical necessity.
GREEN Preventive + problem E/M same day Billing both 99395 (preventive) and 99213 (problem) without modifier 25; reimbursement reduced.	LOW Postpartum care beyond 6 weeks Billing beyond standard 6-week postpartum period without medical necessity documentation.	LOW Global period billing Billing E/M for related follow-up during 90-day surgical global without appropriate modifier.	LOW IUD device not billed 58300 insertion billed without J7307 device; device cost lost.



OIG Work Plan 2025-2026 explicitly lists global OB package double-billing and well-woman exam miscoding as active audit targets. Medicare RACs have recovered > \$140M from OB/GYN practices since 2020.



CLOSING · KEY TAKEAWAYS

Three Actions to Capture OB/GYN Revenue

For OB/GYN practice administrators and revenue cycle directors leading the next 90 days.

01



Deploy global OB package tracker

Integrate EHR with pregnancy tracking. Auto-flag every claim during active global OB period (59400/59510). Require modifier 24/25 justification before claim release. Target: < 3% global OB double-billing rate (industry avg 14%).

🚩 TARGET < 3% double-billing rate

02



Automate modifier 51/59 for GYN surgery

Audit last 200 GYN surgical claims for modifier 51 (multiple procedures) and 59 (distinct procedural service) appropriateness. Implement pre-submission rules engine. Target: < 5% modifier denial rate. Each recovered claim = \$1,850 average.

🚩 TARGET < 5% modifier denial rate

03



Verify infertility coverage before scheduling

Build coverage verification workflow at IVF scheduling. Cross-reference state mandate coverage (19 states). Implement cash-pay workflow for non-mandate state patients. Pre-auth denial rate drops from 24% to < 8% — recovering ~\$102K per 200 IVF cycles.

🚩 TARGET < 8% pre-auth denial rate

Questions & Discussion

Optimizing OB/GYN Revenue Across E/M, Global OB, Surgery & Infertility

✉ support@aetherahealth
care.com

🌐 aetherahealthcare
.com

📞 +1 (863) 694-
0325